



HEALTH ADMINISTRATION PROCEDURES (610) **NATIONAL 2025**

PART 1 Concept Knowledge

Multiple Choice: 25 @ 2 points each _____ (50 points)

PART 3 Production

HPIP _____ (100 points)

SOAP _____ (100 points)

TOTAL POINTS _____ (250 points)

Test Time: 60 minutes

GENERAL GUIDELINES:

Failure to adhere to any of the following rules will result in disqualification:

1. Member must hand in this test booklet and all printouts if any.
2. No equipment, supplies, or materials other than those specified for this event are allowed in the testing area. No previous BPA tests and/or sample tests (handwritten, photocopied, or keyed) are allowed in the testing area.
3. Electronic devices will be monitored according to ACT standards.

Patient Name: Sarah Hamp

PCP: Devin Jones, M.D.

Date of Birth: 10/05/1963

Date of Exam: mm/dd/yyyy *(Today's Date)

Sex: F

HISTORY OF PRESENT ILLNESS: The patient is a 61-year-old white female patient with a history of severe end-stage COPD presented to the ER with complaints of acute shortness of breath. Patient has a long history of COPD and smoking and has continued to smoke in spite of her respiratory problems. She does take bronchodilators at home but has been fairly noncompliant in her care. She also uses home oxygen. She developed acute-onset dyspnea this morning and was brought to the ER via ambulance. History of a collapsed lung. No history of hypertension or heart disease.

MEDICATIONS: Theo-Dur 300 mg t.i.d., Proventil inhaler 2 puffs q.i.d., and home oxygen.

PHYSICAL EXAMINATION: Blood pressure 128/80, pulse 120. Respirations 28 to 32 per minute. Temperature 97.1.

IMPRESSION: Patient appears in moderate distress secondary to her difficulty with breathing. **HEENT:** Pale with very dry skin and dry mucous membranes. **NECK:** Without JVD or HJR. Impression is Right pneumothorax. Chronic obstructive pulmonary disease, end stage. Question of inferior myocardial infarction.

PLAN: 1. Chest tubes have been inserted by the ER physician and patient admitted to ICU. 2. Dr. Suzanne Martin of Pulmonary and Thoracic Services was called in consult because of incomplete reexpansion of patient's pneumothorax. 3. Patient will be monitored and continued on IV corticosteroids as well as antibiotics until we can reexpand the lung. 4. Evaluate for sclerotherapy to prevent recurrent pneumothorax. Repeat EKG once patient is stable.

Devin Jones, M.D.

DJ:contestant #

D: mm/dd/yyyy (Today's Date)

T: mm/dd/yyyy (Tomorrow's Date)

Patient Name: MarySue Patterson

PCP: Josephine Phillips, M.D.

Date of Birth: 07/09/1987

Date of Exam: mm/dd/yyyy (*Today's Date*)

Sex: F

SUBJECTIVE: This is a pleasant 27-year-old white female who is new to the practice. She presents today medication refills. She was given a script for Patanol by her eye doctor to help with some allergic conjunctivitis issues. She also has been prescribed Metrogel by her dermatologist for acne rosacea. She denies any acute concerns at this time. Patient is due for her annual healthcare maintenance. She is going to have records from her other providers sent here. She is on no other medications at this time. Denies any known medication allergies. She is currently a nonsmoker, quit 5 years ago. No illicit drug exposure. Denies significant chronic medical problems. Patient does have an area on her left arm that she would like looked at by a dermatologist. She describes a small mole that has been present for quite some time and may be increasing slightly in size.

OBJECTIVE: Blood pressure 134/86, pulse 85, weight 168 pounds. Focused examination of the left arm reveals a small, circular lesion on the anterior forearm, most consistent with a seborrheic keratosis. This is pigmented and is papular.

ASSESSMENT: Patient is a 27-year-old white female with a history of acne rosacea, followed by Dermatology. She has a script for Metrogel which she would like to have filled. She feels that controls things well. She would like to continue care with Dermatology here. She has a pigmented lesion on her left forearm, most consistent with a seborrheic keratosis. She would like to have this evaluated for possible excision. She will be establishing primary care within our clinic.

PLAN: At this time I have filled her Metrogel and Patanol prescriptions to be continued as current. I have referred her to Dermatology for continued care and further evaluation and management. She will call and arrange that appointment. She will maintain routine follow-up with us and will have her records sent here. She will accomplish her maintenance care after the first of the year. She will return to see me as needed. Return precautions are reviewed. I have reviewed these issues with the patient today. She expresses understanding with the overall plan and agrees to maintain follow-up.

Josephine Phillips, M.D.

JP: contestant #

D: mm/dd/yyyy (*Today's Date*)

T: mm/dd/yyyy (*Tomorrow's Date*)